جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساميه عبدالحكيم محمد سليمان
المحافظة : الشرقيه **الحي / المدينة :** بلبيس **العنوان :** ش الشيخ عبدالعظيم
رقم البطاقة : 26707051301425 **رقم التليفون :** 01125658458 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 05-07-1967 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساميه عبدالحكيم محمد سليمان
National ID: 26707051301425

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

شامل DMS
صفحة
توضيحية





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

NHTMRI

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومى للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجى

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: ساميه عبدالحكيم محمد	**Age:** 59years	**Sex:** female
Request type: مبادره صحه المرأة	**Sub request type:**	
Hospital no:	**Pathology no.:**274B-2026	
Department:	**Date of result:** 6/5/26	
Referred by: د/ايمان عادل	**Date of biopsy:** 29/4/26	
Nature of specimen: Core biopsy	**I.D. Number:**26707051301425	

Clinical diagnosis: Right breast mass

Gross pathology : Multiple gray white tissue cores up to, 1cm long. Totally submitted.

Immunohistochemical technique.

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- Ki67

Microscopic picture: **Microscopic picture:** Section examined revealed core of mammary tissue show malignant growth formed of tubules, nests and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells showing moderate anaplasia and moderate tendency to form tubules. There is moderate number of mitotic figures. There is no ductal carcinoma insitu in core tissue examined. No lymphovascular invasion or perineural infiltration in core tissue examined.

Immunohistochemical Result;

ER: Strong Positive nuclear staining in 90% of tumor cells (Allred Score 8/8).
PR: Moderate Positive nuclear staining of 50% of tumor cells (Allred Score 6/8).
Her2: Moderate incomplete membranous staining of > 10% of tumor cells (Score 1+) Her2 low
Ki67: Positive nuclear staining of 40% of tumor cells.

Diagnosis: Right breast mass; Cor biopsy:
Invasive duct carcinoma (NST), grade II

Signature:
Dr.Nermine Nabil.M.D

Dr. 10/5/2026

Pathology Lab. Manager

Nermine Nabil, M.D

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: سامیه عبدالحکیم محمد سلیمان	
MR. : 10957	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 59-year-old individual with a newly identified retro-areolar breast mass that is described as very hard. Imaging and clinical assessment indicate a T2, N2 lesion with involvement of level 1 and 2 lymph nodes. A core biopsy was performed (M:FREE BX:RT) confirming invasive ductal carcinoma (IDC) of luminal B subtype. A CT of the chest, abdomen, and pelvis was requested for further staging.

Multidisciplinary discussion in July 2026 recommended placement of a port-a-cath and two cycles of examination by ultrasound, followed by neoadjuvant chemotherapy consisting of four cycles of AC (doxorubicin and cyclophosphamide) and then four cycles of Taxotere. Subsequent radiotherapy to the breast and regional nodes (RT MRM) was planned. In August 2026 the patient's ejection fraction was 70% (weight 67 kg, height 165 cm, surface area 1.7 m²). The patient was started on the C1 AC protocol.

Post-radiotherapy assessment shows persistent retro-areolar mass measuring 29 × 23 mm with involvement of the right axillary lymph node. Pathology confirms IDC, grade 2, estrogen receptor 8/8 positive, progesterone receptor 6/8 positive, HER2-negative, and Ki-67 40 %. The disease is staged as cT2 N2. The patient is receiving neoadjuvant AC chemotherapy, currently at cycle 2 (c2 AC).

تحريرا في ,

التاريخ :	20-09-2026	الوقت :	

الطبيب المعالج : ميار فؤاد علي فؤاد

يعتمد ,

Patient ID: 95053	Patient Name English: samia Abdalhakim Mohamed
Age: ٥٨ سنة, ٩ شهر	2026/04/28 12:56:41 PM :Report Date

Bilateral Full field Digital Mammography

Clinical summary: 59y/o complaining of right breast lump, no family history.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Fairly dense parenchyma (ACR density B).
- An irregular retroareolar density is noted with microcalcifications seen within, another UOQ irregular density is noted.

Ultrasound Right breast:

- The retroareolar region shows an irregular retroareolar spiculated hypoechoic lesion measures 29x20mm in dimensions , a pathological IMNL with extracapsular invasion is seen 10-11 o'clock zone c measures 11.9x12.5mm in dimensions with marked vascularity seen within by color Doppler
- Normal skin/nipple/areolar complex.
- Multiple level I and II pathological LN with diffuse cortical thickening the largest 20mm with diffuse cortical thickening 9 mm and 23.8mm with diffuse cortical thickening 6.5mm.

Left breast

- Fairly dense parenchyma (ACR density B).
- No asymmetry, distortion or suspicious clusters of microcalcifications.
- A left UOQ IMLN is noted.

Ultrasound left breast:

- Preserved fibro glandular parenchyma.
- No other solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 5): locally advanced retro-areolar breast lesion with pathological UOQ, level I, level II LNS as described.

Left breast (BIRADS 2): nonspecific IMLN.

Recommendation: Tru cut core needle biopsy was done on the same setting

Best regards,

Dr. Omneya.Ahmed.MD .

Mammogram
[[DOCTYPE:MAMMOGRAM]]

جوستاف روسي

مبـــادرة رئيس الجمهورية
لـدعـم صحة المرأة المصرية

البنك المصرية لتنمية الصحة مصر

mm/dd/yyyy

MDT FORM

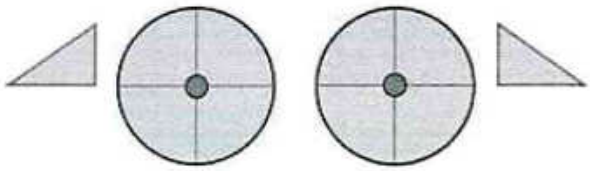
Name:	ساميه عبدالحكيم محمد سليمان	**MRN:**	10957
Age:	59 Y	**ID:**	
Menopausal Sta	POST	**Phone:**	01125531397
NRS SCORE:	PS 1	**Marital Status:**	MARRIED
BMI:	25	**Family History:**	-VE
CO Morbidity:	NAD	**Reason Of MDT:**	NEW CASE

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 6/8 **HER2:** +1 **KI67:** 40

R L



TNM

Sonomammography:
RT LOCALLY ADVANCED RETROAREOLAR BREAST
WITH MULTIPLE LVEL I,II RT AXILLARY
RETRACTED NIPPLE
DELTO PECTORAL LNS

Mets Workup:
CT CAP : FREE

History:

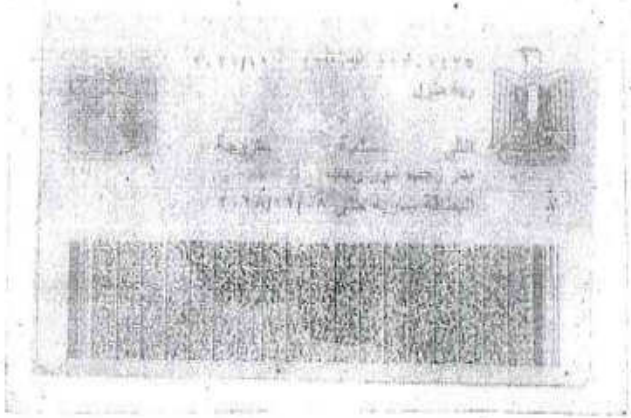
Recommendations:
FOR NACT THEN RT MRM

MDT Outcome & Plan
PORTA CATH INSERTION AND EXM /2CYCLES BY US
FOR NACT(4 CYCLES X AC >> 4CYCLES X TAXOTERE) THEN RT MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د محمد كيلاني / د ندي حسن	د محمد التيراوي				

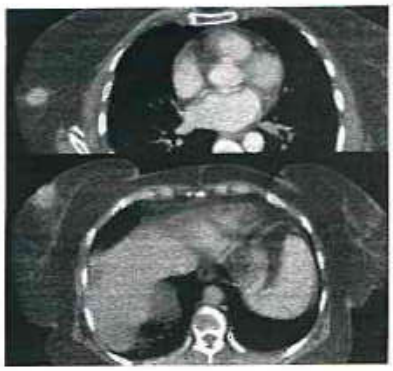




ıe:	**Samia Abdulhakim Mohammed**
Date:	**2026-06-02**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

POST CONTRAST CT SCAN OF THE CHEST , ABDOMEN AND PELVIS WITH CORONAL RECONSTRUCTION: -

i. Chest:



- Right breast shows retro-areolar well-defined enhancing soft tissue mass with nipple retraction. It measures 2 x 1.8 cm. No definite invasion of the pectoralis major muscle is identified.
- Multiple pathologically enlarged right axillary and deltopectoral lymph nodes are noted, showing rounded configuration, loss of fatty hilum, and heterogeneous enhancement, consistent with metastatic nodal disease. The largest measures about 2 x 1.8 cm.
- No left axillary, supraclavicular, or internal mammary lymphadenopathy.
- Lung parenchyma is clear with no focal nodules or masses.
- No pleural effusion.
- No mediastinal or hilar lymphadenopathy.

ii. Abdomen:

- Liver is enlarged with average enhancement with no focal lesions.
- Gallbladder, biliary tree, spleen, pancreas, adrenal glands, and kidneys are unremarkable.
- No abdominal or retroperitoneal lymphadenopathy.

iii. Pelvis:

- Pelvic organs appear unremarkable.
- No pelvic lymphadenopathy.
- IUD is seen in place.

iv. Bones:

- No focal lytic or sclerotic lesions to suggest osseous metastasis.

Impression:

- **Right breast retroareolar neoplastic lesion with retracted nipple.**
- **Multiple metastatic right axillary and deltopectoral lymph nodes.**
- **Hepatomegaly.**

Yours,
Dr. Yahya Khalifa ,Msc,FEBR

NHTMRI

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومى للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجى

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: سامیه عبدالحکیم محمد	**Age: 59years**	**Sex:** female
Request type: مبادره صحه المرأة	**Sub request type:**	
Hospital no:	**Pathology no.:**274B-2026	
Department:	**Date of result:** 6/5/26	
Referred by: د/ایمان عادل	**Date of biopsy:** 29/4/26	
Nature of specimen: Core biopsy	**I.D. Number:**26707051301425	

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Diagnosis: Right breast mass; Cor biopsy:
Invasive duct carcinoma (NST), grade II

Signature:
Dr.Nermine Nabil.M.D

Dr. 10/5/2026

Pathology Lab. Manager

Nermine Nabil, M.D

Patient ID: 95053	Patient Name English: samia Abdalhakim Mohamed
Age: ٥٨ سنة, ٩ شهر	2026/04/28 12:56:41 PM :Report Date

Bilateral Full field Digital Mammography

Clinical summary: 59y/o complaining of right breast lump, no family history.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Fairly dense parenchyma (ACR density B).
- An irregular retroareolar density is noted with microcalcifications seen within, another UOQ irregular density is noted.

Ultrasound Right breast:

- The retroareolar region shows an irregular retroareolar spiculated hypoechoic lesion measures 29x20mm in dimensions , a pathological IMNL with extracapsular invasion is seen 10-11 o'clock zone c measures 11.9x12.5mm in dimensions with marked vascularity seen within by color Doppler
- Normal skin/nipple/areolar complex.
- Multiple level I and II pathological LN with diffuse cortical thickening the largest 20mm with diffuse cortical thickening 9 mm and 23.8mm with diffuse cortical thickening 6.5mm.

Left breast

- Fairly dense parenchyma (ACR density B).
- No asymmetry, distortion or suspicious clusters of microcalcifications.
- A left UOQ IMLN is noted.

Ultrasound left breast:

- Preserved fibro glandular parenchyma.
- No other solid or cystic lesions.
- Normal skin/nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 5): locally advanced retro-areolar breast lesion with pathological UOQ, level I, level II LNS as described.

Left breast (BIRADS 2): nonspecific IMLN.

Recommendation: Tru cut core needle biopsy was done on the same setting

Best regards,

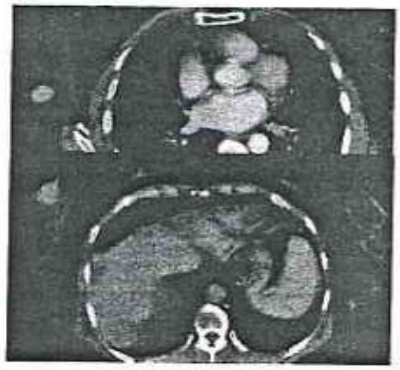
Dr. Omneya.Ahmed.MD .

جوستاف روسي

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Date:	**2026-06-02**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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- No left axillary, supraclavicular, or internal mammary lymphadenopathy.
- Lung parenchyma is clear with no focal nodules or masses.
- No pleural effusion.
- No mediastinal or hilar lymphadenopathy.

ii. Abdomen:

- Liver is enlarged with average enhancement with no focal lesions.
- Gallbladder, biliary tree, spleen, pancreas, adrenal glands, and kidneys are unremarkable.
- No abdominal or retroperitoneal lymphadenopathy.

iii. Pelvis:

- Pelvic organs appear unremarkable.
- No pelvic lymphadenopathy.
- IUD is seen in place.

iv. Bones:

- No focal lytic or sclerotic lesions to suggest osseous metastasis.

Impression:

- **Right breast retroareolar neoplastic lesion with retracted nipple.**
- **Multiple metastatic right axillary and deltopectoral lymph nodes.**
- **Hepatomegaly.**

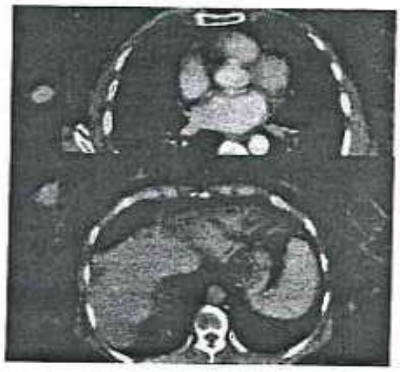
Yours,
Dr. Yahya Khalifa ,Msc,FEBR

جوستاف روسي

ıe:	**Samia Abdulhakim Mohammed**
Date:	**2026-06-02**
Referred by:	Prof.Dr. **no doctor**
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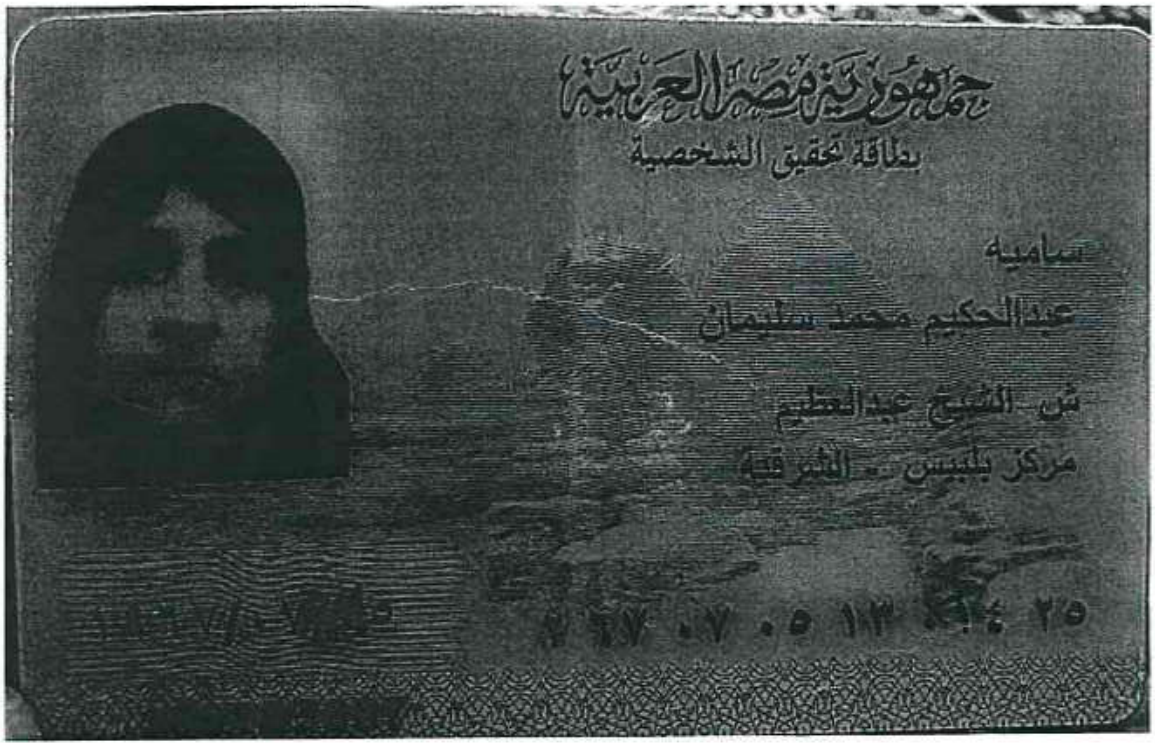
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Impression:

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- **Multiple metastatic right axillary and deltopectoral lymph nodes.**
- **Hepatomegaly.**

Yours,
Dr. Yahya Khalifa ,Msc,FEBR

DMS
مقطع
تدعيم

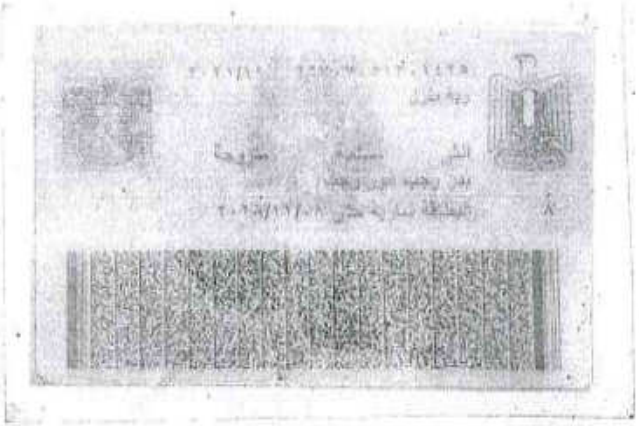





JC8103007







DMS
تأشيرة
مقطعية

Patient ID: 95053	Patient Name English: samia Abdalhakim Mohamed
Age: ٥٨ سنة, ٩ شهر	2026/04/28 12:56:41 PM :Report Date

Bilateral Full field Digital Mammography

Clinical summary: 59y/o complaining of right breast lump, no family history.

Technique

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Detailed Findings

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Left breast

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Ultrasound left breast:

- Preserved fibro glandular parenchyma.
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Impression:

Right breast (BIRADS 5): locally advanced retro-areolar breast lesion with pathological UOQ, level I, level II LNS as described.

Left breast (BIRADS 2): nonspecific IMLN.

Recommendation: Tru cut core needle biopsy was done on the same setting

Best regards,

Dr. Omneya.Ahmed.MD .

NHTMRI

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومى للكبد والجهاز الهضمي
والأمراض المعدية

قسم الباثولوجى

PATHOLOGY DEPARTMENT
HISTOPATHOLOGICAL REPORT

Patient name: ساميه عبدالحكيم محمد	**Age:** 59years	**Sex:** female
Request type: مبادره صحه المرأة	**Sub request type:**	
Hospital no:	**Pathology no.:**274B-2026	
Department:	**Date of result:** 6/5/26	
Referred by: د/ايمان عادل	**Date of biopsy:** 29/4/26	
Nature of specimen: Core biopsy	**I.D. Number:**26707051301425	

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PR: Moderate Positive nuclear staining of 50% of tumor cells (Allred Score 6/8).
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Ki67: Positive nuclear staining of 40% of tumor cells.

Diagnosis: Right breast mass; Cor biopsy:
Invasive duct carcinoma (NST), grade II

Signature:
Dr.Nermine Nabil.M.D

Dr. 10/5/2026

Pathology Lab. Manager

Nermine Nabil, M.D

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مئة مليون صحة

الاسم: سامية عبد الحليم محمد سليم التاريخ:

الرقم القومي: ٢٩٠١٢٠١٤٢٠٥١٢

رقم التليفون: ٠١١٢٥٥٢١٢٩٧

مستهدفة بالتوعية ○

مستهدفة بالتوعية والفحص الإكلينيكي ○

الوزن	الطول	مؤشر كتلة الجسم	قياس السكر العشوائي	قياس ضغط

الإدارة العامة للأشعة
عيادة صحة المرأة
بمعهد الأورام

٣/١١
لم يتم الرد

منطقة المعادي الطبية
مركز صحة الأسرة
بالخبيري

• فكري نفسك باستمرار بموعد الفحص الذاتي شهريًا

أبريل 2024		مايو 2024		يونيو 2024	
يوليو 2024		أغسطس 2024		سبتمبر 2024	
أكتوبر 2024		نوفمبر 2024		ديسمبر 2024	
يناير 2025		فبراير 2025		مارس 2025	
أبريل 2025		مايو 2025		يونيو 2025	
يوليو 2025		أغسطس 2025		سبتمبر 2025	
أكتوبر 2025		نوفمبر 2025		ديسمبر 2025	
يناير 2026		فبراير 2026		مارس 2026	

- حافظي علي قياس السكر والضغط مرة كل ٦ شهور.
- إذا كنتي في سن الزواج أو الإنجاب، إستشيري بإنتظام طبيب تنظيم الأسرة.
- إطمني ، الكشف المبكر بيساعد في السيطرة على الأمراض، وزيادة نسب الشفاء.

موعد الفحص القادم: توقيع الطبيب:

المكان المحولة له للتقييم: موعد التقييم:

ست المصرية هي صحة مصر

منظمة الصحة العالمية
جمهورية مصر العربية

وزارة الصحة والسكان

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